

PHYSICIAN CLINICAL DOCUMENTATION — SOAP NOTE

Patient Name:	Alex Morgan	Patient ID:	P001
Date of Birth:	04/12/1982 (Age 44)	Date of Service:	September 24, 2026
Insurance Plan:	Horizon Blue Cross PPO	Coverage Status:	ACTIVE
Attending:	Dr. Sarah Vance, MD (Spine & Pain Medicine, NPI: 1982736451)	Facility:	Metro Spine & Musculoskeletal Institute, Suite 400

[S] SUBJECTIVE (HISTORY & SYMPTOMATOLOGY)

Chief Complaint: Severe progressive lower back pain with right L5 dermatomal radiation and paresthesias for 10 weeks.

History of Present Illness (HPI): Patient is a 44-year-old female presenting with 10 weeks of unremitting lower back pain following lifting an object. Pain radiates down right posterior gluteal region into lateral calf and dorsum of foot (VAS score 7/10). Pain exacerbated by sitting and lumbar forward flexion; partially alleviated by lying supine with knee flexion. Patient denies bowel or bladder dysfunction, saddle numbness, or systemic symptoms. Activities of daily living and ambulation significantly impaired.

[O] OBJECTIVE (EXAM & PHYSICAL THERAPY RECORD)

Vital Signs: BP 124/78 mmHg, HR 72 bpm, Temp 98.4°F, BMI 24.2.

Musculoskeletal Exam: Antalgic gait favoring right lower extremity. Marked tenderness to palpation over L4-L5 and L5-S1 spinous interspaces with moderate right paraspinal muscle spasm. Lumbar flexion limited to 40° (normal > 60°), extension 10° with reproduction of radicular pain.

Neurological Exam: Motor: 5/5 bilaterally in quadriceps and gastrocnemius; right extensor hallucis longus (EHL) 4+/5. Sensory: Hypoesthesia to light touch and pinprick over right L5 dermatome. Reflexes: Patellar 2+ bilateral; Achilles 1+ on right, 2+ on left. Special Tests: **Straight Leg Raise (SLR) positive on right at 45°**; negative on left.

Documented Conservative Therapy: Completed **8 weeks of supervised physical therapy** (Apex Physical Therapy, 2x/week, Aug 1 - Sep 24, 2026) including core stabilization, McKenzie extension, and pelvic traction. Minimal functional improvement noted.

[A] ASSESSMENT (DIAGNOSES & PRIOR AUTH POLICY EVALUATION)

Primary Diagnoses:

- Lumbosacral radiculopathy, right L5-S1 distribution (ICD-10 M54.16)
- Intractable lumbar intervertebral disc disorder with radiculopathy (ICD-10 M51.16)
- Lumbago with sciatica, right side (ICD-10 M54.41)

Prior Authorization Guideline Criteria Evaluation:

- Active Insurance Coverage: **MET** (Horizon Blue Cross PPO active)
- Symptom / Pain Duration Threshold (≥ 6 weeks): **MET** (10 weeks documented)
- Trial of Supervised Physical Therapy (≥ 6 weeks): **MET** (8 weeks documented)

Recommendation: **APPROVE** — All medical necessity criteria for Lumbar Spine MRI are fully satisfied.

[P] PLAN (IMAGING STUDY, INTERVENTIONS & ORDERS)

- 1. Diagnostic Imaging:** Prior Authorization requested for **Lumbar Spine MRI without contrast (CPT 72148)** to delineate disc herniation and nerve root impingement.
- 2. Medication:** Continue Meloxicam 15mg daily; start Gabapentin 300mg TID for neuropathic pain control.
- 3. Activity:** Gentle walking permitted; avoid heavy lifting > 10 lbs.
- 4. Follow-up:** Return to clinic immediately following MRI acquisition for evaluation and surgical/interventional pain management discussion.

Electronically Signed By:

Dr. Sarah Vance, MD (Spine & Pain Medicine, NPI: 1982736451) • Date: September 24, 2026 15:42 EST